

Manual Test Result Entry

Lab Review Guide

Drax Hall Medical Laboratory · For lab director & senior technologist review

Prepared for: Drax Hall Medical Laboratory

Purpose: Review the information fields staff will use when entering test results manually in the laboratory system.

Please: Check each section against your current bench worksheets and SOPs. Mark anything that needs to be added, removed, or changed.

Date: September 2026

Note: This is a working draft. Custom entry forms were built from common laboratory practice. Your feedback will be used to finalize the system before go-live.

1. What this document covers

Some tests are run on the four main analyzers (Sysmex, Mindray, iFlash, ProLyte). Those results arrive automatically.

Other tests must be entered by hand on the **Bench** screen. This guide lists:

- Tests that have a **custom entry form** (specific fields and dropdown choices)
- Tests that use a **simple general form** (one result line plus optional units and reference range)
- A **checklist** for you to sign off or request changes

2. How staff enter manual results

1. Open the patient's accession on **Bench**.
2. Find the test under **Manual results** (or similar pending area).
3. Click **Enter** or **Edit**.
4. Fill in the form and save.

The result then appears on the patient record, release queue, and exported reports.

3. General entry form (used for most manual tests)

When a test does **not** have a custom form below, staff see these fields:

What staff see	Required?	Examples
Result value	Yes	12, 0 Positive, No growth, Non-Reactive
Units	No	mm/hr, %, mg/dL
Flag	No	Normal, High, Low, Critical, Unknown
Reference range — low	No	Numeric lower limit
Reference range — high	No	Numeric upper limit

4. Custom entry forms — please review these carefully

These tests have **dedicated forms** in the system today. Please confirm the field names, choices, and reference ranges match how you work on the bench.

4.1 ESR (Erythrocyte Sedimentation Rate)

What staff enter	Type	Required?	Units	Normal range
ESR rate	Number	Yes	mm/hr	0 – 20

- The system sets **Normal / High / Low** automatically from the range above.
- **Example on report:** 12

☐ Correct as shown ☐ Changes needed: _____

4.2 Group & Rh (Blood typing)

What staff enter	Type	Required?	Choices
Blood group	Dropdown	Yes	A, B, AB, O
Rh factor	Dropdown	Yes	Rh Positive, Rh Negative

- **Example on report:** Group A, Rh Positive

☐ Correct as shown ☐ Changes needed: _____

4.3 Sickie test (Sickle solubility)

What staff enter	Type	Required?	Choices
Sickle solubility result	Dropdown	Yes	Negative, Positive

☐ Correct as shown ☐ Changes needed: _____

4.4 Direct Coombs (COOMBS – DCT)

What staff enter	Type	Required?	Choices
Direct Coombs result	Dropdown	Yes	Negative, Positive

☐ Correct as shown ☐ Changes needed: _____

4.5 Indirect Coombs (COOMBS – ICT)

What staff enter	Type	Required?	Choices
Indirect Coombs result	Dropdown	Yes	Negative, Positive

☐ Correct as shown ☐ Changes needed: _____

4.6 Urinalysis – Complete

This test requires **two separate entries** before the case can be released:

Part A — Urine chemistry / dipstick

What staff enter	Required?	Choices / notes
Appearance	No	Free text — e.g. Clear, yellow
Protein	No	Negative, Trace, 1+, 2+, 3+, 4+
Glucose	No	Negative, Trace, 1+, 2+, 3+, 4+
Ketones	No	Negative, Trace, 1+, 2+, 3+, 4+
Blood	No	Negative, Trace, 1+, 2+, 3+, 4+
Leukocytes	No	Negative, Trace, 1+, 2+, 3+, 4+
Nitrite	No	Negative, Positive
Additional notes	No	Any other strip observations

☐ Correct as shown ☐ Changes needed: _____

Question for lab: Should we also include pH, specific gravity, urobilinogen, bilirubin, or other strip parameters?

Part B — Urine microscopy

What staff enter	Required?	Examples
WBC per high-power field	No	0–2
RBC per high-power field	No	0–1
Epithelial cells	No	Few
Bacteria	No	None seen
Casts	No	None
Crystals	No	None
Microscopy notes	No	Other sediment findings

☐ Correct as shown ☐ Changes needed: _____

4.7 WBC / Diff — Blood film review

Note: The CBC and automated differential come from the Sysmex analyzer. This form is for the **manual blood film review** that may also be required.

What staff enter	Type	Required?	Choices
Film review finding	Dropdown	Yes	No significant abnormality; Left shift; Toxic granulation; Atypical lymphocytes; Blasts present; Other (see comment)
Comment	Notes box	No	Optional morphology comment

☐ Correct as shown ☐ Changes needed: _____

Question for lab: Is film review required on **every** WBC/Diff order, or only when reflex criteria are met?

5. Sign-off checklist — custom forms

☐	Test	Part	OK?	Changes needed
☐	ESR	—		
☐	Group & Rh	—		
☐	Sickle test	—		
☐	Direct Coombs	—		
☐	Indirect Coombs	—		
☐	Urinalysis — Complete	Chemistry / strip		
☐	Urinalysis — Complete	Microscopy		
☐	WBC / Diff	Blood film review		

Reviewer name: _____ **Date:** _____

Signature: _____

6. All other manual tests (general form)

The tests listed below currently use the **general entry form** (Section 3). If any of these should have their own custom form with multiple fields, please note that in Section 8.

Haematology

CROSS MATCH · FIBRINOGEN · INDICES · LUPUS ANTICOAG · MONO · PT / INR · PTT

Blood Chemistry

A/G Ratio · ALCOHOL · ALK PHOS · AMYLASE · BILIRUBIN (TOT & DIRECT) · CALCIUM · CHOLINESTERASE · GGTP · hs CRP · LDH · LIPASE · LIPO ELECTROPHORESIS · MAGNESIUM · PHOSPHORUS · PROTEIN ELECTROPHORESIS · PROTEINS (Alb & Glob) · URIC ACID

Cardiac Enzymes

LDH

Endocrinology

BHCG QUAL · BHCG QUANT · CORTISOL AM · CORTISOL PM · DHEA-S · ESTRADIOL · INSULIN (0hr, 1hr, 2hr, 3hr) · PROGESTERONE · PSA Free · PSA TOTAL · T3 Free · T3 Total · T3 Uptake · TESTOSTERONE · TESTOSTERONE Free

Immunology

ANA · Anti-DNA · ASLO · ASTO · C3 · C4 · CMV IgG/IgM · CRP · DENGUE IgG/IgM · FEBRILE AGGLUTININS · HAV IgM · HB Core Ab · HBeAg · HBsAb · HBsAg · HCVAb · HERPES I/II IgG/IgM · HIV Confirmation · HTLV I/II · MHA-TP · RA · RUBELLA IgG/IgM · TOXOPLASMA IgG/IgM · VDRL · WIDAL

Anaemia

B12 & FOLATE · G6PD · Hb ELECTROPHORESIS · IRON / TIBC / SATURATION

Special Chemistry

BLOOD LEAD · CA-125 · CEA

Urine Chemistry

17 KGs - 24 hr · 17 Ks - 24 hr · 5SHIAA - 24hr · AMYLASE (urine) · CALCIUM - 24 hr · CREATININE CLEAR - 24 hr · CREATININE - 24 hr / spot · MICROALBUMIN 24 hr · MICROALBUMIN/CREAT RATIO · PHOSPHORUS - 24 hr · PREG TEST (BHCG) · PROTEIN 24hr / spot · VMA - 24 hr

Bacteriology

AFB - SMEAR & CULTURE · CHLAMYDIA · CULT & SENS: FUNGAL · CULT & SENS: ROUTINE · GRAM SMEAR · SPECIMEN · WET PREP

Faeces / Miscellaneous

AMOEBAS · OCCULT BLOOD · OVA & PARASITES · PAP SMEAR · SEMEN ANALYSIS · STONE ANALYSIS

Therapeutic Drug Monitoring

DILANTIN / PHENYTOIN · PHENOBARBITAL · SALICYLATES · TEGRETOL / CARBAMAZEPINE · VALPROIC ACID · OTHER

7. Send-out tests (reference laboratory)

Drugs-of-abuse panels are sent to an outside reference lab. Staff enter the **reference lab result** using the general form (one result value plus optional units and flag).

8. Tests that may need custom forms later

These tests often have **multiple result lines** on paper worksheets. Please indicate if you would like dedicated entry forms for any of them:

Test	Why a custom form may help
SEMEN ANALYSIS	Volume, count, motility, morphology
CULT & SENS: ROUTINE	Organism, colony count, sensitivities
CULT & SENS: FUNGAL	Organism, sensitivities
PT / INR	PT seconds and INR together
BILIRUBIN (TOT & DIRECT)	Total and direct bilirubin
PROTEINS (Alb & Glob)	Albumin and globulin separately
IRON / TIBC / SATURATION	Three related values
B12 & FOLATE	Two separate results
Hb ELECTROPHORESIS	Fraction pattern
CROSS MATCH	Compatibility result and method

- ☐ **None needed — general form is fine for now**
☐ **Custom forms requested — see notes below**

Notes:

9. How to return feedback

For each test that needs changes, please provide:

1. **Test name**
2. **What should staff see on screen** (field labels)

3. **Required or optional**
4. **Dropdown choices** (if applicable)
5. **Units and reference range** (if applicable)
6. **How it should read on the final report**

Return to: _____ **Email / phone:** _____

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